

Anticoagulation: INR Monitoring and Drug Interactions

This is a synthetic, simplified reference document created for a portfolio software project. It is not real clinical guidance and must not be used for patient care.

Patients on warfarin require regular INR monitoring, with frequency depending on stability of control: as often as twice weekly during initiation or dose changes, decreasing to every four weeks once a stable therapeutic range is achieved.

A therapeutic INR range of 2.0-3.0 is typical for most indications, though certain conditions such as mechanical heart valves may require a higher target range. An INR reference range of 0.8-1.1 describes a patient not on anticoagulation; a critical result above 5.0 indicates a substantially elevated bleeding risk and warrants urgent clinical assessment.

Several commonly prescribed medications meaningfully affect warfarin's anticoagulant effect. Aspirin combined with warfarin increases bleeding risk through an additive rather than pharmacokinetic mechanism and should be avoided unless the combination is specifically indicated. Amiodarone inhibits warfarin metabolism and typically requires a dose reduction of roughly one-third to one-half when co-administered, with closer INR monitoring during the transition.

Any new medication, herbal supplement, or significant dietary change (particularly involving vitamin K intake) in a patient on warfarin should prompt a follow-up INR check rather than waiting for the next scheduled interval.